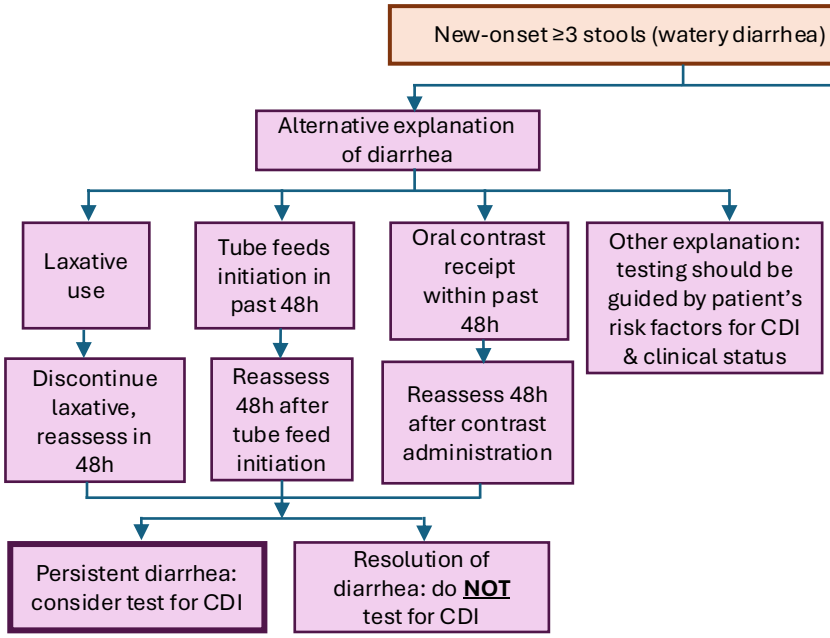
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Diagnostic Stewardship for *C. difficile* at Michigan Medicine

Patients can be infected or colonized with *C. difficile* (*C. diff*). Available assays cannot differentiate between *C. diff* infection or colonization. For FY25, 34% of patients identified with *C. diff* infection did not meet criteria for diagnostic testing. Laxative use and <3 documented diarrheal stools were the most common causes for why *C. diff* tests were ordered inappropriately.

Refer to the C. difficile Treatment Guideline for treatment and testing guidance (see algorithm below).

Indications for *C. diff* Testing



Additional indications for *C. diff* testing:
- ✓ Colostomy output above baseline in 24h without alternate explanation
- ✓ Leukocytosis and ileus suspected to be caused by *C. diff*
- ✓ Acute abdomen with bowel thickening AND diarrhea or ileus
- ✓ Toxic megacolon or evidence of pseudomembranes

***C. diff* testing is NOT recommended for:**
- X Formed stool (patient is asymptomatic)
- X Diarrhea within 48h of laxative use, initiation of tube feeds, or oral contrast
- X Mild post-infectious IBS
- X As a test of cure
- X Currently on therapy for *C. difficile*
- X Within a 14-day period of a positive test
- X Within a 7-day period of a negative test
- X Children <1 year of age

Isolation precautions: Patients with a positive *C. diff* PCR and antigen test require Contact Precautions - Diarrheal for duration of admission.

C. diff Testing Methods:

- **Gastrointestinal Panel PCR plus *C. difficile* by PCR and Toxin bundle:** Recommended for pathogen identification of community-onset diarrhea on hospital days 1-3. Infectious Disease (ID) approval required if ordering on or after hospital day 4, given that the pathogens on this panel are unlikely causes of hospital onset diarrhea, even for immunocompromised patients.
- ***C. difficile by* PCR and Toxin:** Recommended for work up of hospital onset diarrhea (on or after hospital day 4), as this is the principle infectious etiology of hospital onset diarrhea.

Built-in MiChart Clinical Decision Support Tools		Recommended Action
***C. diff* Testing Warning OPA**	Alerts when a *C. diff* test is ordered for a patient admitted >72h AND has had one of the following in the previous 48h: initiation of new tube feedings, oral contrast, or laxatives.	Unless there is high clinical concern for *C. diff* infection (profuse watery diarrhea or acute ileus with abdominal pain and fevers), it is recommended to wait48h after last administration before assessing for ongoing diarrhea and determining the need for *C. diff* testing.
***C. diff* Stool Documentation OPA**	Alerts when a *C. diff* test is ordered for a patient admitted >72h who has not met the minimum stool production frequency, quality, or volume based on documentation in the chart.	Confirm patient's stool frequency, quality, and volume. Discontinue *C. diff* test if stool documentation is accurate and there is not an alternative reason for testing.
Gastrointestinal Panel PCR Restriction	Any GI PCR panel that is ordered on or after day 4 of admission requires approval by the ID consult service.	If approved for GI PCR, enter the name of the ID physician consulted in the order.
***C. diff* by PCR and Toxin Re-testing Restriction**	Following a positive *C. diff* PCR result, MiChart will not allow ordering an additional test for 14 days.	Clinical decision making should consider the original test result and patient's signs and symptoms for acute infection.
***C. diff* Auto Discontinue Orders**	Gastrointestinal Panel PCR, C. diff by PCR and Toxin orders will be discontinued if stool is not collected within 24h of order placement.	

June 2026

ANTIMICROBIAL STEWARDSHIP
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Test Interpretation Guidance for Patients with Compatible Clinical Presentation

PCR Result	Antigen Result	Toxin Result	Interpretation	Recommendation
Negative	Not applicable	Not applicable	No C. diff present	Treatment: Not recommended, the negative predictive value of this test for ruling-out C. diff-associated diarrhea approaches 99% **Contact Precautions-D: Not required**
Positive	Negative	Negative	Low level C. diff colonization	Treatment: Not recommended **Contact Precautions-D: Not required**
Positive	Positive	Negative	C. diff infection or colonization may be present	Treatment: Clinically correlate to determine if treatment is warranted. Consider treatment with severe, non-resolving, or otherwise unexplained diarrhea strongly suggestive of C. diff infection **Contact Precautions-D: Yes, required for rest of admission**
Positive	Positive	Positive	C. diff infection present	Treatment: Recommended **Contact Precautions-D: Yes, required for rest of admission**

NOTE: Hyperlinks from document available here

- C difficile Treatment Guidelines
- Contact Precautions - Diarrheal

Last revised: 06/2026	**Antimicrobial Stewardship Subcommittee:**	**P&T:**

References	
	Citation

Revision History	
Date	**Revision**
06/2026	Updated for revised diagnostic approach

The recommendations in this guide are meant to serve as treatment guidelines for use at Michigan Medicine facilities. If you are an individual experiencing a medical emergency, call 911 immediately. These guidelines should not replace a provider's professional medical advice based on clinical judgment or be used in lieu of an Infectious Diseases consultation when necessary. As a result of ongoing research, practice guidelines may change from time-to-time change. The authors of these guidelines have made all attempts to ensure accuracy based on current information, however, due to ongoing research, users of these guidelines are strongly encouraged to confirm the information contained within them through an independent source